

Bipolar and Depressive Disorders

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Reinvent the Future.

Session Objectives

Describe the etiology and epidemiology of depressive and bipolar disorders.

Compare and contrast the *DSM-V* diagnostic criteria for depressive and bipolar disorders.

List the common differential diagnoses and comorbid conditions for each disorder.

Describe the evidence-based assessments and treatment approaches for each disorder.



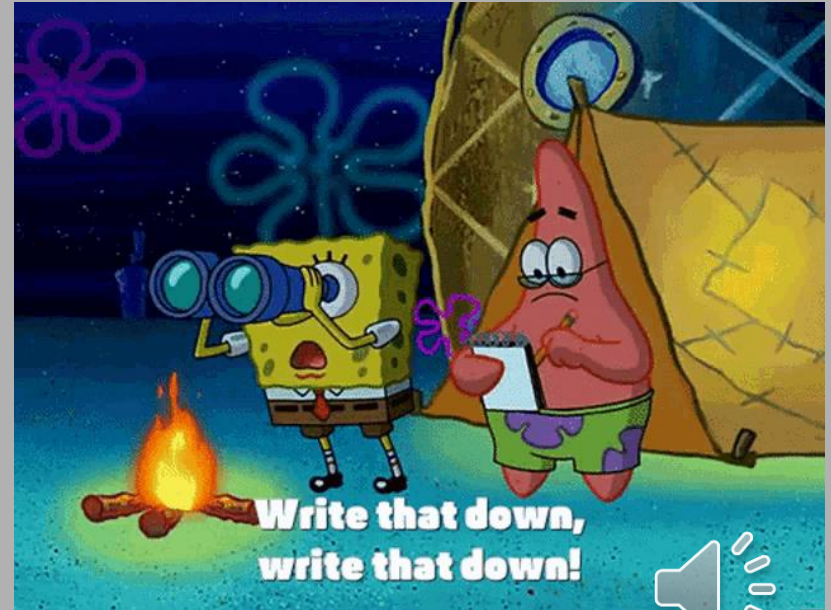
Quick Note:

This lecture will go over high yield topics in Psychiatry that will show up on your board exams. The goal should **NOT** be to simply memorize the DSM-5 criteria and buzz-words, but instead **understand the overall clinical picture**. Very important things you should know will be marked with a **star!**



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Write that down,
write that down!



Clinical Case I

- Nami, a 35 y/o female presents to your clinic for a with complaints of "feeling down". She reports a 2-3 month history of low mood, poor concentration at work, tearful episodes, low energy, feelings of guilt and shame, and a listlessness which is uncharacteristic of her. She denies suicidal thoughts but reports unintentional weight-loss of 10 pounds.



Clinical Case 2

- Luffy, a 52 y/o male presents to your with complaints of “feeling off”. They note a 6-month history of anhedonia, scattered thoughts, a short temper, and random emotional outbursts. which have greatly affected their daily life. They have begun overeating due to stress, and note a 15 pound weight gain. They avoid eye contact when talking and their affect appears flat. They deny thoughts of hurting themselves or others, but state that they have attempted to take their own life once in the past when they were 20 years old.



Question 1

What medical problems can **mimic depressive disorders?**

- A. hypothyroidism
- B. Substance abuse (intoxication or withdrawal)
- C. Low vitamin D
- D. Anemia
- E. Low vitamin B 12
- F. All of the above





Question Break I

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What medical problems can **mimic depressive disorders?**

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- E. Low vitamin B 12
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Clinical Pearl

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Keep in mind, for mental health disorders: you **MUST** rule out other medical conditions first (ex. are symptoms due to depression or are the depressive symptoms due to hypothyroidism?).

In other words, their symptoms are NOT better accounted for by another underlying medical condition! (cancer, substance use, etc.)



Differential for Depressive Symptoms

- Hypothyroidism
- Vitamin deficiencies such as low vitamin B12, folate, vitamin D
- Adjustment disorder
- Prodrome for psychotic disorder
- Depressive episode of bipolar disorder
- Substance use, sometimes active intoxication (alcohol) sometimes withdrawal (stimulants)

Do **NOT** memorize this list, you'll learn all about all these diagnoses in other lectures later!



Differential for Depressive Symptoms

Diagnostics:

- Thyroid functioning tests
- Vitamin B 12 and folate level
- Vitamin D level
- Assess for substance abuse (history and/or lab tests)
- Complete metabolic profile
- Complete Blood Count (CBC)

Do **NOT** memorize this list, you'll learn all about all of these tests and their functions in other lectures later on!



Major Depressive Disorder

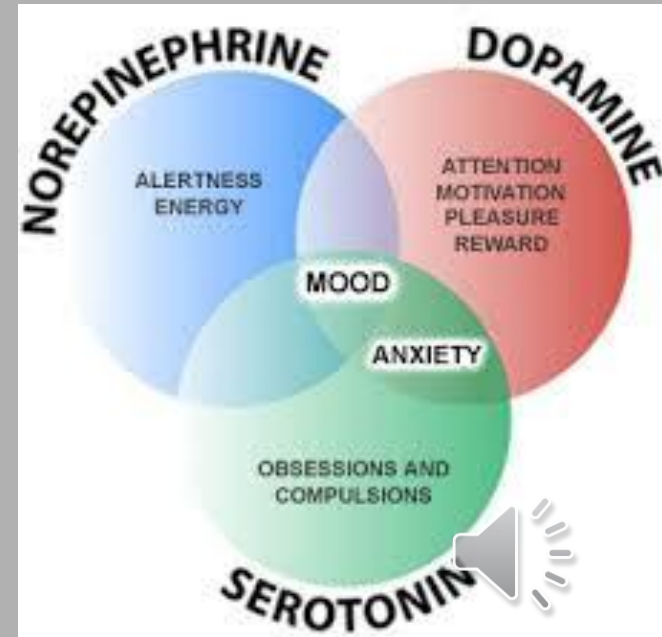
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Pathophysiology

- Depression is typically associated with a **decrease** in neurotransmitters **norepinephrine, serotonin, and dopamine** neurotransmitter levels.
- Brain imaging studies shows a **decrease** in frontal lobe and hippocampal volumes in untreated depression.



Major Depressive Disorder

Diagnosis

DSM-V Criteria



- Symptoms are present most days over the **past 2 weeks**
- **Must impair functioning:** if not, then it is not a disorder
- **Not** better accounted for by another illness or substance (rule out other medical diagnoses and substances first)



Major Depressive Disorder

Epidemiology:

- US lifetime prevalence of MDD is 17%
- Age of onset
 - Peak age of onset for unipolar disorder is 20-40 years old
 - Peak age of onset for psychotic depression is 50-70 years old



Major Depressive Disorder

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DSM-V Criteria: 5 or more of the following:



- Depressed mood
- Decreased interest
- Decreased weight or appetite
- Insomnia/hypersomnia
- Psychomotor agitation or retardation
- Fatigue or loss of energy
- Worthlessness or guilt
- Poor concentration or indecisiveness
- Recurrent thoughts of death



Major Depressive Disorder

Pneumonic to help: **SIGECAPS** describes the *common symptoms* of major depressive disorder (MDD)-



Sleep: Changes in sleep patterns, such as insomnia or excessive sleeping

Interest: Loss of interest or pleasure in activities previously enjoyed (i.e. anhedonia)

Guilt: Feeling guilty or worthless

Energy: Decreased energy levels or fatigue

Concentration: Difficulty concentrating or making decisions

Appetite: Changes in appetite, either increased or decreased

Psychomotor: Agitation (racing mind, fidgeting, shaking of leg, etc) or retardation (slowing down to a crawl, inability to focus or dial in)

Suicide: Thoughts or attempts at suicide



Major Depressive Disorder

Use the Patient Health Questionnaire 2 (PHQ-2) as a screening tool on all your patients! Can be part of review of symptoms.

PHQ-2 Questions

Over the last 2 weeks how often have you been bothered by any of the following problems?	Not at all	Several days	More than half the days	Nearly every day
Little interest or pleasure in doing things	0	1	2	3
Feeling down, depressed, or hopeless	0	1	2	3

- A cut-off score ≥ 3 is positive



Major Depressive Disorder

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PATIENT HEALTH QUESTIONNAIRE-9 (PHQ-9)				
Over the last 2 weeks, how often have you been bothered by any of the following problems? (Use "✓" to indicate your answer)	Not at all	Several days	More than half the days	Nearly every day
1. Little interest or pleasure in doing things	0	1	2	3
2. Feeling down, depressed, or hopeless	0	1	2	3
3. Trouble falling or staying asleep, or sleeping too much	0	1	2	3
4. Feeling tired or having little energy	0	1	2	3
5. Poor appetite or overeating	0	1	2	3
6. Feeling bad about yourself — or that you are a failure or have let yourself or your family down	0	1	2	3
7. Trouble concentrating on things, such as reading the newspaper or watching television	0	1	2	3
8. Moving or speaking so slowly that other people could have noticed? Or the opposite — being so fidgety or restless that you have been moving around a lot more than usual	0	1	2	3
9. Thoughts that you would be better off dead or of hurting yourself in some way	0	1	2	3

FOR OFFICE CODING 0 + _____ + _____ + _____
=Total Score: _____

If you checked off any problems, how difficult have these problems made it for you to do your work, take care of things at home, or get along with other people?

Not difficult at all	Somewhat difficult	Very difficult	Extremely difficult
☐	☐	☐	☐

PHQ-2 Positive?
Use PHQ-9!

<https://www.uspreventiveservicestaskforce.org/Home/GetFileByID/218>



Major Depressive Disorder



Treatment:

First line treatment is **SSRI's and CBT** or other forms of therapy.

- **SSRIs:** Citalopram, escitalopram (lexapro), sertraline (zoloft), paroxetine, fluoxetine. Side effects include: gastrointestinal symptoms, tremor, sedation, jitteriness, headache, insomnia, rash, dry mouth, sexual side effects
- **SNRI's:** Duloxetine (Cymbalta) and venlafaxine
- **NRI:** Bupropion(welbutrin) **contraindicated** in individuals with eating and seizure disorders

You'll learn more about these medications in the dedicated pharmacology lectures!



Major Depressive Disorder



Therapy:

- **Cognitive Behavioral Therapy (CBT)**, interpersonal, psychodynamic, short term therapy, or Acceptance and Commitment Therapy (ACT). All great forms of talk therapy!
- **CBT** is based on several core principles, including:
 - Psychological problems are based, in part, *on faulty or unhelpful* ways of thinking.
 - Psychological problems are based, in part, on *learned patterns of unhelpful behavior*.
 - People suffering from psychological problems can **learn better ways of coping with them**, thereby relieving their symptoms and becoming more effective in their lives.



Major Depressive Disorder



Therapy cont. :

Severe, **treatment resistant depression?** Need a **rapid treatment response?** Try **Electroconvulsive therapy (ECT)!**

- Rapid-acting method to treat refractory depression, depression with psychotic symptoms, catatonia, and acute suicidality.
- **Induces tonic-clonic seizure** under anesthesia and neuromuscular blockade (sounds scary, but it is VERY safe!)
- Adverse effects include disorientation, headache, partial anterograde/retrograde amnesia usually resolving in 6 months.
- No absolute contraindications! **Safe in pregnant individuals and older adults too!**



Major Depressive Disorder

Therapy cont. :

Transcranial Magnetic Stimulation (TMS) uses magnetic fields to stimulate nerve cells in the brain to improve mood and treat treatment-resistant depression that hasn't responded to other therapies, like antidepressants or psychotherapy.

- The non-invasive procedure involves daily, 30-40 minute sessions over several weeks, during which a device on the scalp delivers magnetic pulses to underactive brain areas that regulate mood.
- Can drive yourself to and from treatments unlike ECT
- **Contraindications** include history of seizures or epilepsy as well as electronic implants (intracranial metallic or magnetic hardware)



Clinical Pearl

Why is it so important that we rule out bipolar disorder first?

- Most antidepressants, especially when prescribed first, can worsen manic/hypomanic episodes! (big example of this are SSRIs given as a monotherapy for a patient with bipolar disease).
- However, cases do exist where you can combine a mood stabilizer **AND** an SSRI under physician supervision!



Mood Disorder Questionnaire (MDQ)

Name: _____ Date: _____

Instructions: Check (✓) the answer that best applies to you.
Please answer each question as best you can.

	Yes	No
1. Has there ever been a period of time when you were not your usual self and...		
...you felt so good or so hyper that other people thought you were not your normal self or you were so hyper that you got into trouble?	☐	☐
...you were so irritable that you shouted at people or started fights or arguments?	☐	☐
...you felt much more self-confident than usual?	☐	☐
...you got much less sleep than usual and found you didn't really miss it?	☐	☐
...you were much more talkative or spoke faster than usual?	☐	☐
...thoughts raced through your head or you couldn't slow your mind down?	☐	☐
...you were so easily distracted by things around you that you had trouble concentrating or staying on track?	☐	☐
...you had much more energy than usual?	☐	☐
...you were much more active or did many more things than usual?	☐	☐
...you were much more social or outgoing than usual, for example, you telephoned friends in the middle of the night?	☐	☐
...you were much more interested in sex than usual?	☐	☐
...you did things that were unusual for you or that other people might have thought were excessive, foolish, or risky?	☐	☐
...spending money got you or your family in trouble?	☐	☐
2. If you checked YES to more than one of the above, have several of these ever happened during the same period of time? <i>Please check 1 response only.</i>	☐	☐
3. How much of a problem did any of these cause you — like being able to work; having family, money, or legal troubles; getting into arguments or fights? <i>Please check 1 response only.</i>		
☐ No problem ☐ Minor problem ☐ Moderate problem ☐ Serious problem		
4. Have any of your blood relatives (ie, children, siblings, parents, grandparents, aunts, uncles) had manic-depressive illness or bipolar disorder?	☐	☐
5. Has a health professional ever told you that you have manic-depressive illness or bipolar disorder?	☐	☐

This questionnaire should be used as a starting point. It is not a substitute for a full medical evaluation. Bipolar disorder is a complex illness, and **an accurate, thorough diagnosis can only be made through a personal evaluation by your doctor.**

Adapted from Hirschfeld R, Williams J, Spitzer RL, et al. Development and validation of a screening instrument for bipolar spectrum disorder: the Mood Disorder Questionnaire. *Am J Psychiatry*. 2000;157:1873-1875.

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Mood Disorder Questionnaire

Best at screening for bipolar I disorder, less sensitive for bipolar II and unspecified bipolar disorder. Remember, it is **screening tool, not a diagnostic tool**



The American Journal of Psychiatry <https://doi.org/10.1176/appi.ajp.160.1.178>
<https://www.integration.samhsa.gov/images/res/MDQ.pdf>

Back to our Two Cases...

- Here are some test results and further history for both of our patients:
- Mood Disorder Questionnaire (MDQ) and history are both **negative** for previous hypomania/mania episodes
- Lab results: all values within normal limits
- No recent loss or remarkable change in life 

Question 2

Based on the clinical picture presented what diagnosis do both Luffy and Nami have?

- A. Adjustment Disorder with Depressed Mood
- B. Persistent Depressive Disorder
- C. Major Depressive Disorder
- D. Minor Depressive Disorder



Question Break 2

Based on the clinical picture presented what diagnosis do both Luffy and Nami have?

- A. Adjustment Disorder with Depressed Mood
- B. Persistent Depressive Disorder
- C. Major Depressive Disorder
- D. Minor Depressive Disorder



Question 3

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True or False: Asking
about suicide puts the
idea in the patient's
mind



Massachusetts General Hospital Comprehensive Clinical Psychiatry, Chapter twenty-nine: Epidemiology
of major depressive disorder

Question 3

True or False: Asking about suicide put the idea in the patient's mind

- **False**, asking about suicide thoughts does **NOT** increase suicidal thoughts
- 21% of patients with recurrent depressive disorder attempt suicide.
- We can remember the risk factors for suicide with the mnemonic **SAD PERSONS**



Risk Factors Mnemonic

Risk factors for suicide death **SAD PERSONS** are more likely to pass from suicide.



- S**ex (male)
- A**ge (young adult or older adult)
- D**epression
- P**revious attempt (*highest risk factor*)
- E**thanol or drug use Rational thinking loss (psychosis)
- S**ickness (medical illness)
- O**rganized plan
- N**o spouse or other social support
- S**tated future intent





Subsets of Depression:

Persistent Depressive Disorder

- Depressed mood for **at least 2 years**. Also called dysthymia. Often milder than MDD; ≥ 2 depressive symptoms lasting ≥ 2 years (≥ 1 year in children), with any remission lasting ≤ 2 months.
- **Treatment:** standard MDD therapies + ECT/TMS is treatment resistant

MDD with seasonal pattern:

- Formerly called seasonal affective disorder. Major depressive episodes **occurring only during a particular season** (usually winter) in **≥ 2 consecutive years** and in most years across a lifetime. Atypical symptoms common.
- **Treatment:** standard MDD therapies + light therapy.



Subsets of Depression:



Depression with atypical features:

- Characterized by mood reactivity (transient improvement in response to a positive event), hypersomnia, hyperphagia, leaden paralysis (heavy feeling in arms and legs), long-standing interpersonal rejection sensitivity (anxious expectation of being rejected). Most common subtype of depression.
- **Treatment:** CBT and SSRIs are first line. MAO inhibitors are effective but not first line because of their risk profile.

Substance/Medication Induced Depressive Disorder

- Common substances include alcohol, opiates, cocaine, amphetamines, and corticosteroids. Symptoms develop soon after intoxication or withdrawal or after medication exposure.
- **Treatment:** supportive measures and discontinue use of substance/medication



Subsets of Depression:



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Depressive Disorder Due to Another Medical Condition

- Possible causes: hypothyroidism, adrenal insufficiency, systemic lupus erythematosus, vitamin B12 deficiency.
- **Treatment:** treat the underlying medical cause



Peripartum mood disturbances:

Onset **during pregnancy** or **within 4 weeks of delivery**.

Increased risk with history of mood disorders.

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Postpartum blues:

- 50–85% incidence rate. Characterized by depressed affect, tearfulness, and fatigue starting 2–3 days after delivery. Usually resolves within 2 weeks.
- **Treatment:** supportive. Follow up to assess for possible MDD with peripartum onset.

MDD with peripartum onset:

- 10–15% incidence rate. Formerly called postpartum depression. Meets MDD criteria with onset either during pregnancy or within 4 weeks after delivery.
- **Treatment:** CBT and SSRIs are first line



Other Mood Disorders

Premenstrual Dysphoric Disorder



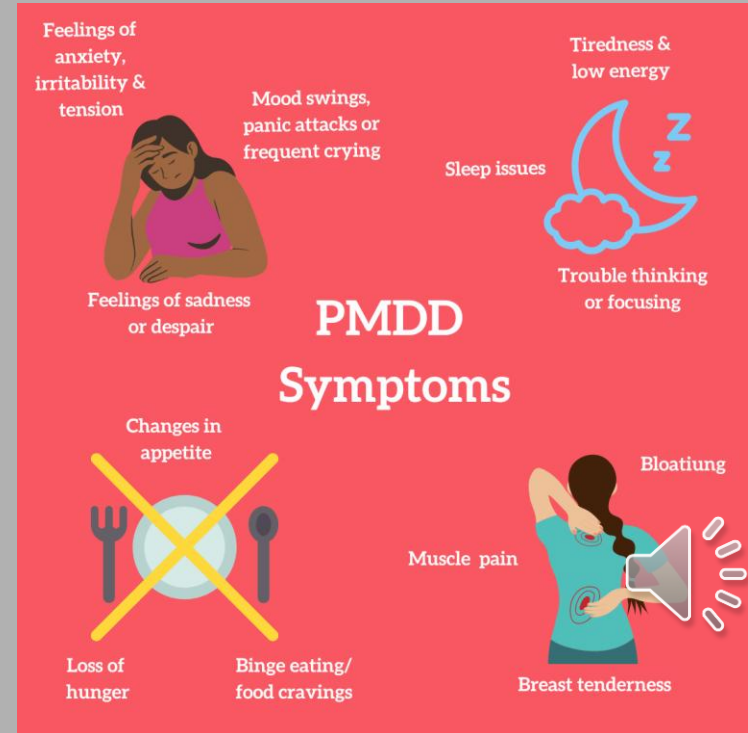
Behavioral and mood symptoms that begin the week **before** the start of menstrual cycle and end within **first few days** of menses

- **Diagnosis:** At least **5 symptoms total, including 1 or more mood symptoms**
- **Treatment:** SSRIs (first line), oral contraceptives, lifestyle changes (regular exercise, stress management, limit caffeine/alcohol, etc), GnRH agonists for severe cases

Conn's Current Therapy 2021, Depressive, Bipolar, and related mood disorders, Section Unipolar Depressive disorders. <https://login.nyit.idm.oclc.org/login?url=http://www-clinicalkey-com.arktos.nyit.edu/#!/content/book/3-s2.0-B9780323790062001804?scrollTo=%23top>

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Other Mood Disorders

Premenstrual Dysphoric Disorder cont.

Mood Symptoms:

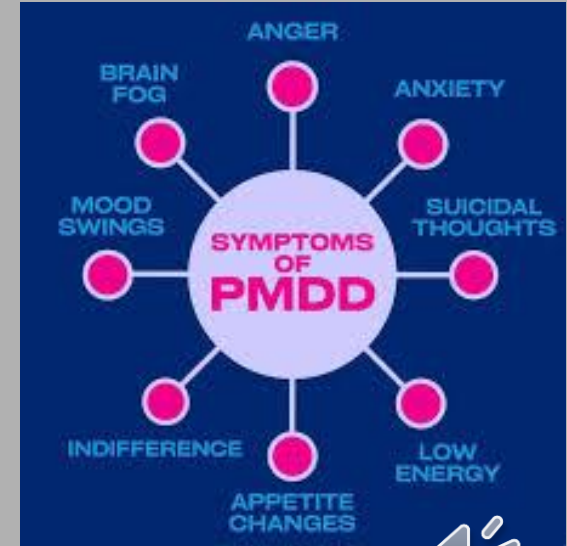
- mood swings (suddenly sad, tearful, sensitive to rejection)
- irritability or anger,
- depressed mood,
- Hopelessness

Other Symptoms:

- Decreased interest in usual activities
- Difficulty concentrating
- Low energy or fatigue
- Change in appetite, overeating, cravings
- Sleep problems (insomnia or hypersomnia)
- Feeling overwhelmed or out of control
- Physical symptoms (breast tenderness, bloating, joint/muscle pain)

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Other Mood Disorders



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Disruptive mood Dysregulation Disorder

Severe, chronic, irritability in children including frequent temper outbursts plus persistent irritable mood

- **Diagnosis: 3 or more** severe temper outbursts (verbal or behavioral), **out of proportion to situation per week**
- Symptoms occur in at **least 2 settings** (e.g., home, school, with peers).
- Symptoms last **at least 12 months** (with **no break >3 months**).
- Diagnosis **made between ages 6–18**. Onset of symptoms **before age 10**.



Other Mood Disorders

Disruptive mood Dysregulation Disorder cont.

- **Treatment:** Cognitive Behavioral Therapy, Parent Management Training, and medications if impairment is severe (stimulants, SSRIs)

Note: **Cannot** coexist with bipolar disorder or intermittent explosive disorder (but can with ADHD, depression, etc.).

Note: Between outbursts, mood is **persistently** irritable or angry most of the day, *nearly every day*.



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Clinical Case 3

- Steve, a 29 y/o male, presents to your clinic for with his partner. He reports anger problems, which he notes are affecting his marriage. He's healthy and on no medications. He denies current or past drug and alcohol use, which is corroborated by his partner. He is pleasant and cooperative, good eye contact, easily engages in conversation. You interview his wife and learn that not only is he irritable, but also more self-confident, starts new projects and hobbies, doesn't sleep much, talks faster, and wants sex all the time for about a 5-6 days at a time. These symptoms are followed immediately by a period of low-mood, decreased energy, and sleeping a lot. MDQ questionnaire was negative.



Clinical Case 4

- Charlotte, a 35 y/o female, presents to ED via EMS. She was brought in after her roommate called 911. According to the roommate, charlotte hasn't slept in 3 weeks, instead choosing to work on a proposal that could stop world hunger. She has also had different sexual partners visit the apartment every night during this time. Her roommate grew increasingly concerned for Charlotte's safety, prompting her to call 911. In the ED, urine tox screen is negative. When Charlotte's family arrives, they note she frequently cycles between periods of intense energy and lethargy. Of note, she had one inpatient stay for attempted suicide at age 27 and takes no medications



Question 4:

What are some of the
key points in these
histories?



Question 4

Steve:

- Episodic anger problems
- Negative for drug or alcohol use
- Half finished projects
- Cycles between high and low energy that lasts ***less than*** a week at a time



Question 4

Charlotte:

- Brought in by EMS
- Negative for drug or alcohol use
- Cycles between high and low energy that lasts ***greater than a week*** at a time
- Previous hospitalization and suicide attempt



Bipolar Disorders

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DSM-V Criteria for Mania

Distinct period of persistently elevated, expansive or irritable mood or increased activity/energy lasting at least one week.

3 or more (or 4 if irritable mood) of the following symptoms are present:

- Inflated self-esteem or grandiosity
- Decreased need for sleep
- Pressured speech
- Racing thoughts or Flight of Ideas
- Distractibility
- Increased activity
- Excess or risky activity



Bipolar Disorders



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DIG FAST is a mnemonic used to help remember the key symptoms of a manic episode, a symptom of bipolar disorder.

- D** – Distractibility- easily lose their attention or constantly jump between tasks
- I** - Indiscretion or impulsivity- excessive involvement in impulsive or reckless behaviors (gambling, sex, substance abuse, fiscally irresponsible purchases, etc.)
- G** – Grandiosity- excessive self esteem or self-image that may progress to delusions of grandeur
- F** - Flight of ideas or racing thoughts- rapid flow of disconnected thoughts.
- A** - Activity Increase or goal-directed behavior- staying up all night to read a book, do homework, binge watch a TV show, etc. Stems from psychomotor agitation.
- S** - Sleep deficit- inability to sleep, feeling tired, insomnia, etc.
- T** – Talkativeness- pressured speech, seemingly unending. Tone varies from swift and loud to heated and hostile

<https://www.osmosis.org/answers/dig-fast-manic-episode-mnemonic>



Bipolar Disorders



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- Differences between Mania and Hypomania
 - Hypomanic episode generally lasts **at least 4 days but less than 7**
 - **Only manic episodes** will get some admitted to psych unit or arrested (not hypomanic episode)
 - People with hypomania are often still functional while hypomanic. As such, hypomania is a bit more difficult to diagnose



Bipolar Disorders



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Bipolar I (requires 1 type of episode):

≥ 1 manic episode with or without a hypomanic or depressive episode (may be separated by any length of time).

Bipolar II (requires 2 types of episodes):

a hypomanic and a depressive episode (no history of manic episodes).

Bipolar disorder I involves mania and depressive symptoms, while Bipolar II involves hypomania and depressive symptoms.

Cyclothymic disorder: milder form of bipolar disorder fluctuating between mild depressive and hypomanic symptoms. Must last ≥ 2 years with symptoms present at least half of the time, with any remission lasting ≤ 2 months.



Bipolar Disorder

Differential includes:

- Schizophrenia or schizoaffective disorder
- Anxiety disorders
- Substance Use disorders
- Borderline Personality Disorder
- Secondary Mania—MS, steroids, brain tumor
- Bipolar Spectrum illness
- Bipolar disorder, current episode manic



Question 5

True or False: If manic symptoms result in hospitalization, it is characterized as mania and not hypomania, regardless of the timing of symptoms.



Question 5

True or False: if manic symptoms result in hospitalization, it is characterized as mania and not hypomania, regardless of the timing of symptoms.

TRUE!

Hypomania = less severe, no hospitalization, no psychosis, lasts at least 4 days but less than 7

Mania = more severe, causes marked impairment, may require hospitalization, may include psychosis, lasts at least 1 week (or any duration if hospitalized).



Bipolar Disorders



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Treatment

- Second generation antipsychotics (SGA's) or Mood stabilizers (valproic acid, lithium, carbamazepine).
- Treatment is now **based on what phase of the illness the person is in**: manic or depressed.
- Therapy to assist with improving insight into mental illness, improving compliance to medications, and to teach the patient to better identify their mood changes.

- **Acute Mania**: SGA and Mood Stabilizers mono or dual therapy
- **Acute Depression**: SGA plus SSRI or other antidepressant (monotherapy or adjunctive to mood stabilizer like lithium).

Note: Sometimes when antidepressants are used as a monotherapy, they can bring on a manic episode/worsen manic episodes! Therefore, if used, they need to be combined with SGA's or Mood Stabilizers



Summary Slide

MDD: Recurrent episodes lasting ≥ 2 weeks characterized by ≥ 5 of 9 diagnostic symptoms including depressed mood or anhedonia (or irritability in children) SIG E CAPS

Persistent depressive disorder: Also called dysthymia. Often milder than MDD; ≥ 2 depressive symptoms lasting ≥ 2 years (≥ 1 year in children), with any remission lasting ≤ 2 months.

MDD with seasonal pattern: Formerly called seasonal affective disorder. Major depressive episodes occurring only during a particular season (usually winter) in ≥ 2 consecutive years and in most years across a lifetime.

Depression with atypical features: Characterized by mood reactivity (transient improvement in response to a positive event), hypersomnia, hyperphagia, leaden paralysis (heavy feeling in arms and legs), long-standing interpersonal rejection sensitivity. Most common subtype of depression.

Summary Slide

Substance/Medication Induced Depressive Disorder: Common substances include alcohol, opiates, cocaine, amphetamines, and corticosteroids. Symptoms develop soon after intoxication or withdrawal or after medication exposure

Depressive Disorder Due to Another Medical Condition: Possible causes: hypothyroidism, adrenal insufficiency, systemic lupus, erythematosis, vitamin B 12 deficiency

Peripartum mood disturbances: Onset during pregnancy or within 4 weeks of delivery. Increased risk with history of mood disorders.

Postpartum blues: Characterized by depressed affect, tearfulness, and fatigue starting 2–3 days after delivery. Usually resolves within 2 weeks.



MDD with peripartum onset: Meets MDD criteria with onset either during pregnancy or within 4 weeks after delivery. Treatment: CBT and SSRIs are first line

Summary Slide

Premenstrual Dysphoric Disorder (PMDD): A severe, cyclic mood disorder where emotional and physical symptoms occur in the luteal phase of the menstrual cycle and resolve after menses, causing significant impairment.

Disruptive Mood Dysregulation Disorder (DMDD): A childhood disorder marked by chronic irritability and frequent, severe temper outbursts disproportionate to the situation, persisting for at least one year.

Bipolar I (requires 1 type of episode): 1 manic episode +/- a hypomanic or depressive episode (may be separated by any length of time).

Bipolar II (requires 2 types of episodes): a hypomanic and a depressive episode (no history of manic episodes).

Cyclothymic disorder: milder form of bipolar disorder fluctuating between mild depressive and hypomanic symptoms. Must last ≥ 2 years with symptoms present at least half of the time, with any remission lasting ≤ 2 months.



References

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- First Aid for USLME 2024
- DSM-5
- Dirty Medicine on YouTube
- Chapter 13: Depressive and Bipolar and related disorders:
<https://nyit.idm.oclc.org/login?url=https://www.clinicalkey.com/#!/content/book/3-s2.0-B9780323833783001816?scrollTo=%23h10000877>
- Massachusetts General Hospital Comprehensive Clinical Psychiatry, Chapter 29, Depressive Disorders & Chapter 30, Bipolar Disorder. <https://nyit.idm.oclc.org/login?url=https://www.clinicalkey.com/#!/browse/book/3-s2.0-C20090325519>
- <https://www.apa.org/ptsd-guideline/patients-and-families/cognitive-behavioral>



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